PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Association of cultural origin and migration status with work-related mental health of migrants and refugees in Europe – A systematic review protocol
AUTHORS	Herold, Regina; Wuchenauer, Frederik; Kandler, Anja; Morawa, Eva; Unverzagt, Susanne; Voss, Amanda; Erim, Yesim

VERSION 1 – REVIEW

REVIEWER	Brunnet, Alice Université Paris-Nanterre, Psychology
REVIEW RETURNED	10-Jun-2021

GENERAL COMMENTS	Thank you for the opportunity of reviewing this study protocol. The authors wish to explore, in a systematic review, the relationship between work-related conditions and mental health of migrants and refugees living in Europe. This is a relevant topic and this systematic review will be an important contribution for the literature. The study protocol is well written, clear and the proposed methods are consistent with a systematic review. Regarding the method section, it is unclear why only three databases will be searched. One of the strengths of the proposed protocol is to include articles written in diverse languages, therefore I suggest that the authors consider including databases in those languages (for example, Cairn.info for articles in French, Lilacs/Scielo for articles in Spanish). Also, it is unclear how authors will “pay special attention” to health workers. I would recommend to the authors to decide whether to do a review including all professions or to do a specific review including only health workers. If they opt for the first option, the “with special consideration to the health care sector” could be removed from the title. If they opt for the second option, additions or changes to the main objective, keywords, and search strategy would be needed. Finally, I would like to give some other insights regarding the analysis of the studies. Regarding the work conditions and migration status, I would also include migrants in irregular migratory situation or working in informal jobs. I understand that these populations are difficult to access and probably few studies have been conducted, however, I would add as points to pay attention to during the studies analysis. As a last point, in addition to the migrants’ countries of origin, a comparison of the results across the different host countries would be beneficial. Taking into account different migration laws in the European countries (for example, the possibility of work while waiting for the asylum application), and the different reactions of the host societies (acceptance of the difference, discrimination, xenophobia, etc).

REVIEWER	Hou, Wai Kai Education University of Hong Kong
REVIEW RETURNED	30-Jul-2021

GENERAL COMMENTS	This systematic review protocol aims to investigate the relationship between work-related conditions and mental health among migrants and refugees in Europe. Three databases will be searched for synthesizing related quantitative studies published since 2016. This study has several strengths. First, the research aims and study protocol are clearly described. Second, the inclusion of studies written in different European languages is an advantage to be able to exhaustively include the relevant literature studying migrants and refugees in Europe. Third, the potential findings emerging from this study carried implications for developing psychotherapies targeted at improving the mental health of migrants and refugees at the workplace. The potential contribution is nonetheless diminished by the following limitations. First, the theoretical framework is not clear. Migrants and refugees/asylum seekers are clearly distinct populations with past traumatic experiences disproportionally high among refugees/asylum seekers. More conceptualization and empirical evidence are needed to establish a close link between cultural origin and migration status and mental health of migrants and refugees and to highlight the empirical and policy implications of comparing these two distinct populations with each other. Second, inclusion criteria need more clarity. 1. For type of population, authors are suggested to discuss whether they will include studies in the following situation: articles do not provide clear information on the generation of migrants, do not provide clear information on the residence status of migrants, or do not separate working age and non-working age participants in their study. 2. For type of exposure, authors are suggested to provide citations for their classification on working conditions (organizational conditions, social conditions at work, special issues such as post-migration stressors at the workplace). 3. For type of outcomes: mental health outcomes are very broad in the current protocol including psychiatric or psychological symptoms, perceived distress, diagnosed psychiatric and psychological disorders, or suicide or attempted suicide. In addition, well-being such as quality of life and life satisfaction are also included. It will be hard to compare between such a broad range of mental health outcomes. The authors are encouraged to provide classifications for mental health outcomes or maybe consider narrowing down their scope. Third, the authors said that they will only include quantitative studies (RCT, cohort studies, case-control, and cross-sectional studies) and exclude qualitative studies and case studies. However, their “Data analysis and synthesis” said that no formal quantitative analysis will be conducted. Authors should justify why only quantitative studies will be included. Considering the complexity of research aims, authors are encouraged to conduct meta-analysis for quantitative synthesis of previous evidence. The later will require thorough descriptions in the study protocol.

VERSION 1 – AUTHOR RESPONSE

Reviewer: 1
Dr. Alice Brunnet, Université de Picardie Jules Verne

Comments to the Author:
Thank you for the opportunity of reviewing this study protocol. The authors wish to explore, in a systematic review, the relationship between work-related conditions and mental health of migrants and refugees living in Europe. This is a relevant topic and this systematic review will be an important contribution for the literature.
The study protocol is well written, clear and the proposed methods are consistent with a systematic review.
5. Regarding the method section, it is unclear why only three databases will be searched. One of the strengths of the proposed protocol is to include articles written in diverse languages, therefore I suggest that the authors consider including databases in those languages (for example, Cairn.info for articles in French, Lilacs/Scielo for articles in Spanish).

Thank you for this important remark. Unfortunately, however, it is not possible for us to search separately relevant databases for each language. This is because we simply cover too many languages. Even before the search we thought about including more databases, but the overlaps with the three most important databases for our research area (PubMed, PsycInfo, CINAHL) were very large, so we decided to use these databases. In addition, some of the databases mentioned, such as LILACS/Scielo, have their focus on South American countries, which does not make sense for us, as our focus is on European countries only. Moreover, Cairn.info covers, for example, many fields of research, with psychology and cultural studies being only a very small part of it (communications, economics, education, geography, history, literature, linguistics, philosophy, political science, law, pychology, sociology, and cultural studies). Nevertheless, we would like to take our advantage and include all articles in the 7 languages we listed from these three databases. In the methods section, information was added that these three databases are the most relevant to this area of research.

6. Also, it is unclear how authors will “pay special attention” to health workers. I would recommend to the authors to decide whether to do a review including all professions or to do a specific review including only health workers. If they opt for the first option, the “with special consideration to the health care sector” could be removed from the title. If they opt for the second option, additions or changes to the main objective, keywords, and search strategy would be needed.

Thank you for this helpful comment. All professions will now be considered equally. The title was changed and all theoretical explanations on this subject have been deleted.

7. Finally, I would like to give some other insights regarding the analysis of the studies. Regarding the work conditions and migration status, I would also include migrants in irregular migratory situation or working in informal jobs. I understand that these populations are difficult to access and probably few studies have been conducted, however, I would add as points to pay attention to during the studies analysis.

Thank you for this important remark. Migrants in irregular migration situations and migrants who are informally employed are also included (see inclusion and exclusion criteria). To make this clearer, this sentence has been added under "Types of population".

8. As a last point, in addition to the migrants' countries of origin, a comparison of the results across the different host countries would be beneficial. Taking into account different migration laws in the European countries (for example, the possibility of work while waiting for the asylum application), and the different reactions of the host societies (acceptance of the difference, discrimination, xenophobia, etc).

Thank you very much for this idea! We will also compare the working conditions and their impact on the mental health of migrants and refugees/asylum seekers living and working in different host countries. Information on this planned comparison was added to the Abstract, Introduction, the section of „Data analysis and synthesis", and Discussion.

Reviewer: 2
Dr. Wai Kai Hou, Education University of Hong Kong

Comments to the Author:
This systematic review protocol aims to investigate the relationship between work-related conditions and mental health among migrants and refugees in Europe. Three databases will be searched for synthesizing related quantitative studies published since 2016. This study has several strengths. First, the research aims and study protocol are clearly described. Second, the inclusion of studies written in different European languages is an advantage to be able to exhaustively include the relevant literature studying migrants and refugees in Europe. Third, the potential findings emerging from this study carried implications for developing psychotherapies targeted at improving the mental health of migrants and refugees at the workplace. The potential contribution is nonetheless diminished by the following limitations.

9. First, the theoretical framework is not clear. Migrants and refugees/asylum seekers are clearly distinct populations with past traumatic experiences disproportionally high among refugees/asylum seekers.

Thank you for this very important remark! Precise definitions of, and thus differences between, subgroups (international migrants, refugees, and asylum seekers) were added to clarify the framework. Reference to the particular vulnerability of refugees and asylum seekers has also been added („Due to the greater psychological strain on refugees and asylum seekers, it is reasonable to assume that this particular subgroup could suffer particularly from poor working conditions.") However, I have already mentioned the most important conceptual factors (= factors based on empirical evidence). („Refugees and asylum seekers can be seen as a specific migrant group because, by definition, they have different pre- and postmigratory conditions due to the sometimes traumatic refugee experience itself and the subsequent asylum process, making them differently vulnerable to mental health problems.")

10. More conceptualization and empirical evidence are needed to establish a close link between cultural origin and migration status and mental health of migrants and refugees and to highlight the empirical and policy implications of comparing these two distinct populations with each other.

Thank you for this important advice. The link between cultural background and mental health and the link between migration and mental health have been more clearly conceptualized using empirical evidence. It was added that it is necessary to consider these two populations separately due to their differences, in order to develop and apply helpful assistance specifically directed to both groups.

11. Second, inclusion criteria need more clarity.

1. For type of population, authors are suggested to discuss whether they will include studies in the following situation: articles do not provide clear information on the generation of migrants, do not provide clear information on the residence status of migrants, or do not separate working age and non-working age participants in their study.

The procedure for missing information as well as for not separating work age and non-work age participants was added.

12. 2. For type of exposure, authors are suggested to provide citations for their classification on working conditions (organizational conditions, social conditions at work, special issues such as post-migration stressors at the workplace).

Thank you for this important remark. Discussions with a group of experts took place during the conceptualization of this classification, after which this categorization was agreed upon. For this reason, no previously conducted study exists with exactly this structure. It is based on expert opinions. Only a citation for the category of "postmigratory stressors" could be added. In order to present the individual categories more clearly, more examples have been added to the individual categories.

13. 3. For type of outcomes: mental health outcomes are very broad in the current protocol including psychiatric or psychological symptoms, perceived distress, diagnosed psychiatric and psychological disorders, or suicide or attempted suicide. In addition, well-being such as quality of life and life satisfaction are also included. It will be hard to compare between such a broad range of mental health outcomes. The authors are encouraged to provide classifications for mental health outcomes or maybe consider narrowing down their scope.

Thank you for this objection. Unfortunately, we do not want to narrow our scope of mental health outcomes. The reason for this is that, in addition to mental disorders/diagnoses that can be established on the basis of clear criteria, simple syndromes or the simple state of health also reflect individuals' well-being and thus mental health. If we were to focus only on diagnoses and ignore the simple question of how satisfied individuals are with their lives or how they assess their quality of life, we would lose a great deal of information about mental health. That is the reason why we have defined our outcomes that broadly.
When extracting and describing the results, it is planned to classify them. Cluster A will consist of psychological and psychiatric diagnoses (including suicide and suicide attempts), cluster B will consist of psychological and psychiatric symptoms (e.g., anxiety, depression, PTSD, somatoform disorders), and cluster C will describe general distress. Cluster D will include more general related constructs of mental health (wellbeing, quality of life, life satisfaction). Cluster E will include all outcomes that cannot be assigned to clusters A-D.
Information about each cluster has been incorporated into the review protocol.

14. Third, the authors said that they will only include quantitative studies (RCT, cohort studies, case-control, and cross-sectional studies) and exclude qualitative studies and case studies. However, their “Data analysis and synthesis” said that no formal quantitative analysis will be conducted. Authors should justify why only quantitative studies will be included.

Thank you for this important advice. However, only quantitative studies will be included, as this allows for better comparability of the studies included. Since the outcome and exposure are already defined quite broadly, it must be ensured that the studies are methodologically similar.

15. Considering the complexity of research aims, authors are encouraged to conduct meta-analysis for quantitative synthesis of previous evidence. The later will require thorough descriptions in the study protocol.

Thank you for this suggestion. However, main aim of this systematic review is a narrative synthesis without meta-analysis of effect size of very different outcomes. If one were to perform meta-analytic calculations on these different constructs, the apples-and-oranges problem would arise. Outcomes from different primary studies would be summarized, which differ with regard to central characteristics such as the operationalization, the exact definition of the construct as well as the measurement instrument. However, these can not be meaningfully compared or related. If we will find sufficient primary studies that have focused on the same construct (using the same measurement instrument), we will decide whether metanalytic calculations are appropriate and meaningful, especially since our primary goal is first to describe the relationship between working conditions and mental health and to describe the working conditions themselves, not metanalytic calculations. Meta-analytical calculations could be performed in the next step for specific outcomes.

Reviewer: 1
Competing interests of Reviewer: I have no competing interests.

Reviewer: 2
Competing interests of Reviewer: None declared.

VERSION 2 – REVIEW

REVIEWER	Brunnet, Alice Université Paris-Nanterre, Psychology
REVIEW RETURNED	22-Nov-2021

GENERAL COMMENTS	The authors have adequately responded to all my questions and made the necessary changes in the manuscript. Therefore, I suggest the publication of this manuscript.